

Diagnosis of Adamantiades-Behçet Disease

Diagnosis of Adamantiades-Behçet Disease is based on a scoring system requiring a total of 3 points:

- **1 point:** Oral aphthosis
- **1 point:** Skin manifestations (pseudo-folliculitis, skin aphthosis)
- **1 point:** Vascular lesions (phlebitis, superficial phlebitis, large vein thrombosis, aneurysm, arterial thrombosis)
- **1 point:** Positive pathergy test
- **2 points:** Genital aphthosis
- **2 points:** Ocular lesions

These criteria were originally established by the Behçet Disease Research Committee of Japan. However, limitations in selectivity and specificity have led to revisions of these diagnostic criteria.

Mucocutaneous Lesions

Recurrent oral and genital aphthous ulcers are the most common mucosal manifestations. Oral aphthous ulcers are the initial manifestation in over 80% of cases. Although recurrent aphthous stomatitis is common, only a small subset of patients develop Adamantiades-Behçet disease, and no reliable predictors exist for identifying progression.

Oral ulcers are typically multiple, painful, 1 to 3 cm in diameter, with well-defined margins, a fibrin-coated base, and surrounding erythema. They usually heal without scarring (92%). Genital ulcers tend to heal with scarring (64% to 88%) and may persist longer than oral lesions. Healing of oral aphthae occurs within 4 days to 1 month.

Large oral ulcers may involve the pharynx, leading to dysphagia, dyspnea, or fistula formation in the pharynx, larynx, trachea, or esophagus. Genital ulcers

can affect the penis, scrotum, vagina, labia, urethra, and may also occur in the anal, perineal, and inguinal regions.

Diagnostically relevant skin lesions in Adamantiades-Behçet disease include: - Pustular vasculitic lesions (including pathergy reactions)

- Erythema nodosum-like lesions
- Sweet-like lesions
- Pyoderma gangrenosum-like lesions
- Palpable purpuric lesions due to necrotizing venulitis

All these lesions show a neutrophilic vascular reaction in early stages. Acneiform or follicle-based pustules are not considered diagnostically relevant.

Systemic Lesions

Ocular involvement is a major cause of morbidity. The most significant diagnostic lesion is **posterior uveitis** (also known as retinal vasculitis), which can result in blindness.

Revised Diagnostic Criteria of the Behçet's Disease Research Committee of Japan

Main Symptoms

- Recurrent oral aphthous ulcers
- Skin lesions
 - Erythema nodosum
 - Superficial thrombophlebitis
- Papules
- Skin hypersensitivity
- Ocular lesions
 - Iridocyclitis or sequelae
 - Posterior uveitis or sequelae
- Genital ulcers

Additional Symptoms

- Arthritis without deformity or sclerosis
- Epididymitis
- Gastrointestinal lesions (e.g., ileocecal ulcerations)
- Vascular lesions
- Central nervous system lesions (moderate or severe)

Diagnostic Categories

- **Complete type:** Presence of four main symptoms
- **Incomplete type:** Three main symptoms, or two main and two additional symptoms, or typical ocular lesions plus one other main symptom or two additional symptoms
- **Suspected disease:** Typical main symptoms not meeting criteria for incomplete type
- **Special lesions:** Specific gastrointestinal, vascular, or central nervous system involvement

Laboratory Findings (Supportive, Not Essential)

- Pathergy test (positive or negative)
- Prick test to streptococcal vaccine (positive or negative)
- Signs of inflammatory response
- Positive HLA-B51
- Other pathological findings

Additional Notes

Non-typical symptoms should not be used to diagnose Behçet disease.